

Housing Insecurity

TC: C-1165 (TC)

[Link to Codes](#)

MCG Health
Transitions of Care
28th Edition

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Assessment

Q1. Patient: Are you worried or concerned that in the next 2 months you may not have stable housing that you own, rent, or stay in as a part of a household?

- Yes [Q2] [B1] [P1]
- No
- Do not currently have housing [Q2] [B1] [P1]
- Unknown [Q2]

Q2. Patient: Do you need help with any of the following? (Check all that apply.)

- Finding housing resources [P1]
- Filling out paperwork/electronic forms [P1]
- Following up on existing referrals [P1]
- Meeting medical needs due to housing insecurity [P1]
- Declined
- Not applicable

Plan of Care

Barriers

B1. Housing inadequate

Problems

P1. Housing insecurity present

- **Goal:** Housing modified or changed so it is adequate, stable, and safe(1)[N](#)
 - **Interventions:**
 - **ASSIST:** Identifying housing options
 - **ASSIST:** Completing documentation for housing assistance, if needed
 - **ASSIST:** Appointment scheduling with provider if housing problems negatively affecting self-care
 - **ASSIST:** Identifying available housing resources and contacting for support
 - **EDUCATE:** Availability of resources and interventions varies by region.
 - **EDUCATE:** Types of housing resources available; examples include(2)(3):
 - Faith-based organizations
 - Subsidized housing
 - Shelters
 - Supportive housing programs[N](#)
 - **EDUCATE: Resources:**
 - Benefit Finder: www.benefits.gov
 - Findhelp: www.findhelp.org
 - United Way: www.211.org
 - **COORDINATE:** Social worker referral, as needed(4)
 - **COORDINATE:** Community services referral, as needed(5)(6)
 - **COORDINATE:** Follow-up contact to evaluate housing status, referral status, and need for further assistance(7)(8)
 - **SEND:** Choosing a place to live [N](#) (Spanish)
 - **Goal:** Medical needs being met while housing insecurity present(9)
 - **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Identifying factors of unstable housing that might influence medical needs (eg, lack of medication storage, hygiene care, food quality, protection from elements during sleep)
- **ASSIST:** Identifying available community and medical resources and contacting for support (eg, food resources, cell phone assistance, street clinics)(10)
- **EDUCATE:** Role of community and medical services to support self-care(6)
- **EDUCATE:** Harm reduction activities (eg, avoid alcohol/drugs, safety plan, sexually transmitted disease education, immunizations)
- **COORDINATE:** Increased intervals of follow-up with preferred method of communication (eg, phone, text, email)(3)
- **COORDINATE:** Social worker referral, as needed(4)
- **COORDINATE:** Community services referral, as needed(5)(6)
- **COORDINATE:** Follow-up contact to evaluate housing status, referral status, and need for further assistance(7)(8)

Evidence Summary

Quality standards: Standards indicate that program content should address psychosocial issues (eg, inadequate or unsafe housing) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(11)

Outcomes: A retrospective analysis of 1248 Medicare beneficiaries indicated that housing with supportive social services has the potential to reduce hospital use and decrease spending for older patients with social needs.(12)

References

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Codes

ICD-10 Diagnosis: Z59.00, Z59.01, Z59.02, Z59.10, Z59.11, Z59.12, Z59.19, Z59.2, Z59.3, Z59.5, Z59.6, Z59.7, Z59.811, Z59.812, Z59.819, Z59.89, Z59.9 [Hide]

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